



+ New On

Print



Ranked By panel

Primary Care - Metabolic

Primary Care - Oncology

Ranked by
percentile

Primary Care - Metabolic

Last updated 10/08/2026

Diabetes Type 2
in 1 year

15.4%
probability

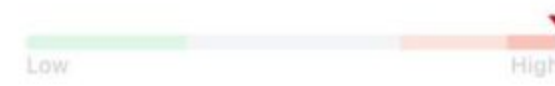


100th
percentile



Chronic Kidney Disease
in 1 year

14.2%
probability



100th
percentile



GLP-1 RA For Diabetes
Care Opportunity

0.69%
probability



94th
percentile



GLP-1 Inhibitors For Weight Loss
Care Opportunity

0.32%
probability



59th
percentile



Metabolic Dysfunction-
associated Steatohepatitis
(MASH)
Screening

0.018%
probability



7th
percentile

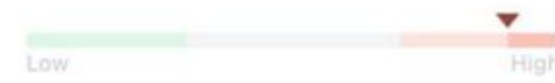


Primary Care - Oncology

Last updated 10/08/2026

All-Cause Hospitalizations
in 1 year

12.3%
probability



90th
percentile



Thomps
MRN 92643

ANEL
bolic
omes



GLP-1 RA For Diabetes

Endocrine · Care Opportunity

Less likely

More likely

More likely than 94% of eligible patients

LIKELIHOOD TO START

Top 10%

most likely to start

COMPARED WITH PEERS

3.5x

vs cohort base rate (0.20%)

MODEL PERFORMANCE

AUC 0.86

model discrimination

ELIGIBLE COHORT

Age ≥18

WHY THIS OPPORTUNITY SURFACED ⓘ

- Type 2 diabetes mellitus without complications · 3 · **4.17x**
- Essential (primary) hypertension · 15 · **1.47x**
- Type 2 diabetes mellitus with hyperglycemia · 2 · **5.67x**
- Lantus SoloStar Subcutaneous Solution Pen-injector 100 UNIT/ · 8 · **8.82x**
- Hemoglobin; glycosylated (A1C) · 2 · **1.56x**

▼ Show all 12

CONSIDERATIONS ⓘ

- Consider diabetes medication regimen review and monitoring for GLP-1 RA appropriateness within the next 3 months.
- Monitor diabetes control parameters (e.g., A1C and renal markers) as part of routine follow-up.

Last assessed 10/08/2026 · stable since last assessment

rformance, and 1

VBC Risk Adjustment

Value-based care (VBC) risk adjustment: undiagnosed and emerging conditions relevant to risk adjustment programs

Diabetes Type 2	365 days	Population (age>=18) No prior history	0.801	Future Risk	Endocrine
Chronic Kidney Disease	365 days	General Population (age >=18) No prior history	0.813	Future Risk	Renal
Dementia	365 days	General Population (age>=61) No prior history	0.840	Future Risk	Neurologic
Heart Failure	365 days	General Population (age>=18) No prior history	0.870	Future Risk	Cardiovascular
Atrial Fibrillation	365 days	Age ≥18, no prior outcome	0.833	Future Risk	Cardiovascular
Pulmonary Fibrosis / Interstitial Lung Disease (ILD)	0 days	age 45+	0.780	Screening	Respiratory

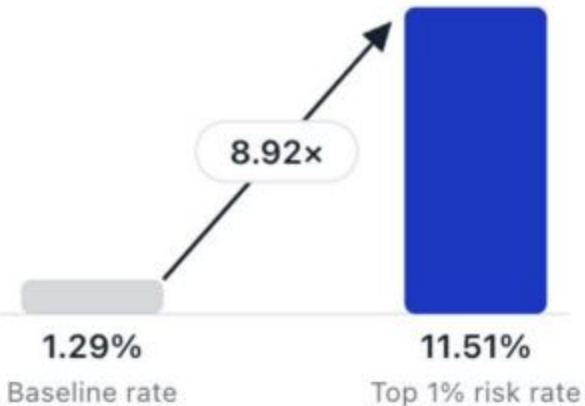
Chronic Kidney Disease



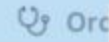
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TIMEFRAME	COHORT
365 days	General Population (Age ≥18) No prior history
MODEL TYPE	SYSTEM
Future Risk	Renal
PANELS	
Primary Care - Metabolic	VBC Risk Adjustment
PERFORMANCE	
AUROC	0.813
Baseline rate	1.29%

TOP 1% RISK VS. BASELINE



Results



Open Slots

Jump to...

October 9, 2026

N	VISIT TYPE	PHENOM RESULT
01280	Follow-Up	Reviewed
74806	Annual Wellness	Results Ready
43976	Follow-Up	Results Ready
00208	Annual Wellness	Results Ready
48166	Follow-Up	Reviewed
06179	Annual Wellness	Results Ready
032708	Follow-Up	Results Ready
055478	Annual Wellness	Results Ready
047770	Follow-Up	Reviewed
009853	Annual Wellness	Results Ready
03140	Follow-Up	Results Ready
003141	Annual Wellness	Results Ready

ML

Martinez, Laura

MRN 5965478 · 66F

Ranked

By panel

Ranked by percentile

All panels

♀	Osteoporosis Screening	33.8% probability	Low High	96th percentile
🔥	Diabetes Type 2 in 1 year	5.0% probability	Low High	94th percentile
🔥	Chronic Kidney Disease in 1 year	3.5% probability	Low High	92nd percentile
♀	Breast Cancer in 1 year	0.37% probability	Low High	91st percentile
💚	Substance Use Disorder Screening	5.8% probability	Low High	88th percentile
💚	Major Depressive Disorder (MDD) Screening	19.3% probability	Low High	86th percentile
🔥	Metabolic Dysfunction-associated Steatohepatitis (MASH) Screening	0.33% probability	Low High	79th percentile
🔥	GLP-1 RA For Diabetes Care Opportunity	0.008% probability	Low High	9th percentile
🔥	Polycystic Ovary Syndrome	1.6%	Low High	6th percentile